Consent, Refusal and the Documentation of Choice

What makes consent valid, who may give it, which consents have their own law, and why the form is not the consent.

Module	06 of 12
Study time	105 minutes
Audience	All clinicians who perform procedures, nursing staff, medical records staff, legal and quality teams
Assessment	Knowledge check plus a 50-question scored bank
Standards basis	NABH 6th Edition PRE chapter; MTP Act 1971 as amended 2021; HIV/AIDS Act 2017; Mental Healthcare Act 2017; THOTA; DPDP Act 2023

Consent is the area where hospitals most often believe they are compliant and most often are not. The forms are present, signed and filed; but the forms record a signature, and what has to be proved is a conversation. This module covers what makes consent valid, who may give it in each situation, the consents that carry their own statutory rules, how to document a refusal, and what a consent audit actually looks for.

LEARNING OUTCOMES — by the end of this module you will be able to:

- State the six elements of valid consent and explain what each requires in the record
- Determine who may lawfully consent in a given clinical situation and document the basis
- Document consent for a procedure so that the conversation, not only the signature, is evidenced
- Apply the specific statutory rules for MTP, sterilisation, HIV testing, transplantation, mental health, research and clinical photography
- Document a refusal of treatment and a discharge against medical advice defensibly
- Audit a sample of consent forms against an objective standard

6.1 What consent is, and what the form is

Consent is a decision made by a person with capacity, on the basis of adequate information, free of pressure, for a specified intervention. The form is the record that this happened. When the two are confused, hospitals produce perfectly completed forms that prove nothing, because the only question a court asks is whether the patient was actually told what a reasonable patient would want to know.

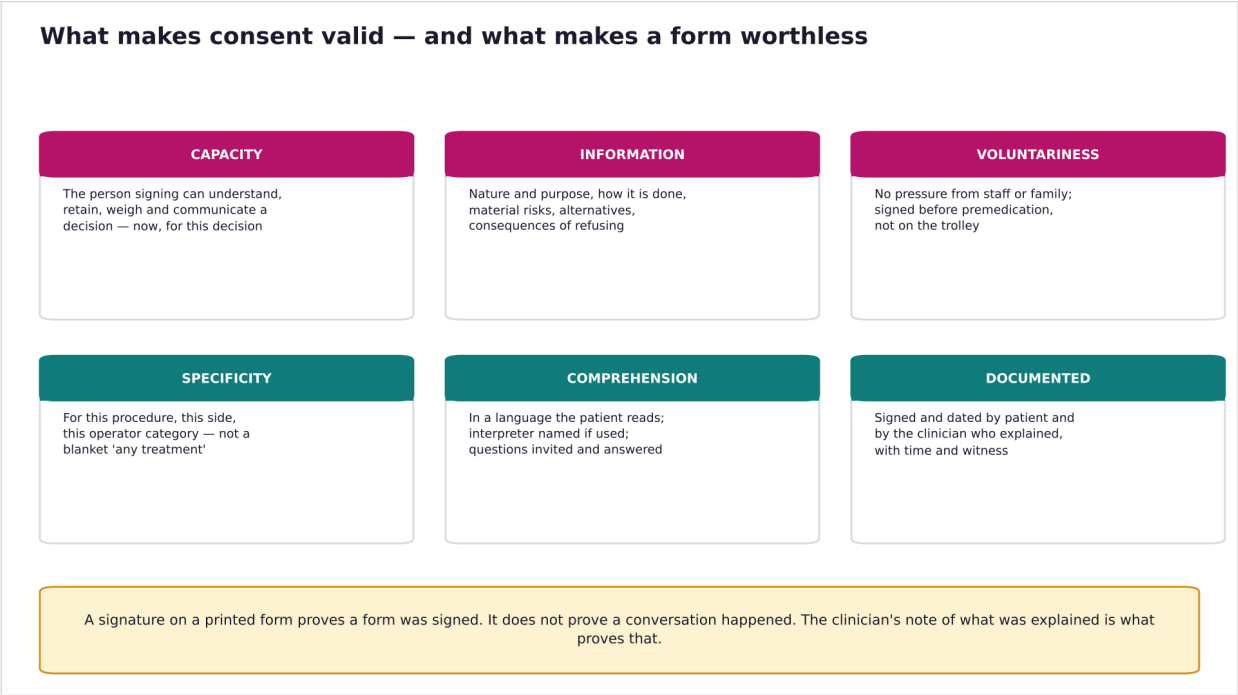


Figure 6.1 — The elements of valid consent, and what each one requires in the record.

KEY POINT
The clinician's own note of the consent discussion — what was explained, in what terms, what the patient asked, and what was decided — is worth more than the signed form on its own. In almost every disputed consent case, the question is not whether a form was signed but what was said.

6.2 Capacity

Capacity is decision-specific and time-specific. A patient may have capacity to consent to a blood test and not to a major amputation; a patient may lack capacity at 3 a.m. while septic and regain it by the next morning. The record should state the basis on which capacity was assessed, not merely assert it.

- Can the patient understand the information relevant to the decision?
- Can they retain it long enough to use it?
- Can they weigh it, including the consequences of deciding either way?
- Can they communicate their decision by any means?

Where capacity is in doubt, the record should state what was assessed and what was concluded, and where capacity is absent, it should state what steps were taken to support the patient to decide — a quieter environment, an interpreter, waiting for sedation to wear off — before moving to a substitute decision-maker.

CAUTION
A patient who disagrees with the recommended treatment does not thereby lack capacity. Capacity is about the process of deciding, not the wisdom of the decision. Recording a patient as lacking capacity because they refused is a serious error and is readily identified as one.

6.3 Information: how much must be disclosed

The standard that Indian courts increasingly apply is what a reasonable patient in that patient's position would consider material, rather than what a body of doctors would customarily disclose. In practice the record should show that the following were covered.

Element	What to document
Nature and purpose of the procedure	What is being done and what it is intended to achieve, in words the patient used or understood
How it will be done	Anaesthesia type, approximate duration, what the patient will experience, expected recovery
Material risks	Both common and less common but serious risks; risks of particular significance to this patient given their occupation, circumstances or stated concerns
Alternatives	Including non-operative management and including doing nothing, with the implications of each
Consequences of refusal	What is likely to happen if the procedure is not performed
Who will perform it	The operator or the category of operator, and whether trainees will be involved
Extensions	What would be done if an unexpected finding arises during the procedure, and the limits of what the patient authorises
Costs, where relevant	Particularly for implants, prostheses and procedures with significant out-of-pocket components

6.4 Who may give consent

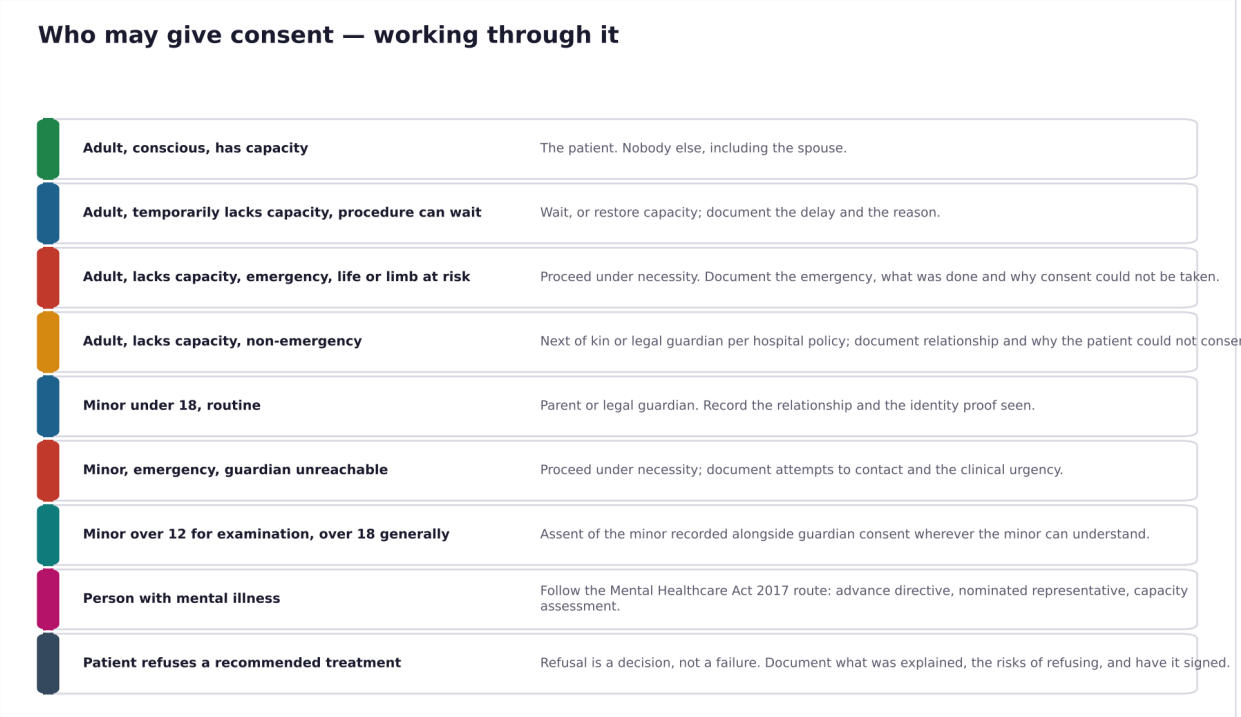


Figure 6.2 — Determining who may lawfully consent, situation by situation.

The most frequent error in Indian hospitals is taking consent from the spouse or an adult son for a conscious adult patient who has capacity. This is culturally common and legally wrong: the patient's own consent is required, and a family member's signature in place of it is not consent at all. Where the patient wishes the family to be involved in the decision, that preference should itself be recorded.

At the other extreme, in a genuine emergency where the patient lacks capacity and delay would risk life or limb, treatment proceeds under the doctrine of necessity. The record must then show the nature of the emergency, why consent could not be obtained, what attempts were made to contact a relative, what was done, and who made the decision.

Situation	Who consents	What the record must additionally show
Adult with capacity	The patient, personally	Nothing further; family consent in place of the patient is a defect
Adult lacking capacity, emergency	No one — treatment under necessity	The emergency, the clinical urgency, attempts to contact relatives, the decision-maker, and the intervention limited to what was necessary
Adult lacking capacity, non-emergency	Next of kin or legal guardian per hospital policy	Why the patient could not consent, the relationship of the person consenting, and the identity proof seen
Minor under eighteen	Parent or legal guardian	Relationship and identity verified; the minor's own assent recorded where they are able to understand
Minor, emergency, guardian not contactable	No one — treatment under necessity	Attempts to contact, the clinical urgency, and the decision-maker
Person with mental illness	Per the Mental Healthcare Act 2017 framework	Capacity assessment, any advance directive, the nominated representative, and the statutory route being followed
Patient under the influence of alcohol, drugs or premedication	Defer if possible; otherwise treat as lacking capacity	That consent was not taken in that state, and either the deferral or the necessity basis
Unidentified unconscious patient	No one — necessity	Identification attempts, police intimation where applicable, and the clinical decisions taken

6.5 When consent is taken, and by whom

Two timing rules carry most of the risk. Consent must be taken before premedication, because a sedated patient cannot give valid consent, and a signature obtained on the trolley outside theatre is routinely challenged. And consent should be taken far enough in advance that the patient has time to consider — for elective procedures this means at the point the decision to operate is made, with confirmation on admission.

Consent should be obtained by the person performing the procedure, or by someone capable of performing it and able to answer questions about it. A consent taken by a nurse or an intern who cannot describe the procedure's risks is a form-filling exercise, and when the patient says "nobody explained it to me", it is difficult to contradict them.

> **CAUTION**
> Where the consent form has a line for the person who explained the procedure, that line names the person who will be asked, years later, what they said. It should never be signed by somebody who was not present for the explanation.

6.6 Consents with their own legal rules

Consents with their own legal rules

Situation	Governing rule	What the record must show
Medical termination of pregnancy	MTP Act 1971 as amended 2021; Form C	Woman's own written consent; guardian only if below 18 or mentally ill; opinion of the required number of RMPs; confidentiality of identity
Sterilisation	Standards for Female and Male Sterilisation; prescribed consent form	Informed that the procedure is permanent, that failure is possible, and that alternatives were offered; spouse's consent is NOT required
HIV testing	HIV/AIDS (Prevention and Control) Act 2017	Informed consent and pre-test counselling recorded; result disclosed only to the person tested unless an exception applies
Organ donation / transplant	THOTA 1994 as amended	Prescribed forms, authorisation committee where applicable, donor and recipient documentation kept separately and securely
Clinical photography, filming, teaching	Hospital policy and DPDP Act 2023	Separate written consent naming the purpose and where the images may appear; right to withdraw recorded
Research participation	ICMR National Ethical Guidelines; ethics committee approval	Approved participant information sheet, version-numbered consent form, ethics approval reference, and a copy given to the participant

A general surgical consent form does not cover any of these. Each needs its own form, filed with the episode it belongs to.

Figure 6.3 — Consents governed by specific statutes, and what each requires.

A general surgical consent form does not cover any of the situations in the figure. Each needs its own form, with its own content, filed with the episode it belongs to. Three deserve particular emphasis because hospitals commonly get them wrong.

Sterilisation

The consent is the patient's own. Spousal consent is not a legal requirement and a hospital that insists on it is acting outside the law. The record must show that the patient was told the procedure is intended to be permanent, that failure is possible even when the procedure is performed correctly, and that reversible alternatives were offered.

Medical termination of pregnancy

The woman's own written consent is required and is sufficient; the consent of a husband or partner is not required. Guardian consent applies only where the woman is below eighteen or is a person with mental illness. The opinion of the number of registered medical practitioners required by the gestational stage must be documented, and the identity of the woman is protected by a statutory confidentiality duty, so the record must be stored accordingly.

HIV testing

Under the HIV and AIDS (Prevention and Control) Act 2017, testing requires informed consent and pre-test counselling, both documented, and the result may be disclosed only to the person tested except in the narrow circumstances the Act allows. Testing a patient without consent — for example as part of a pre-operative panel — is unlawful, and recording the result in a file that is widely accessible compounds the breach.

6.7 Documenting refusal

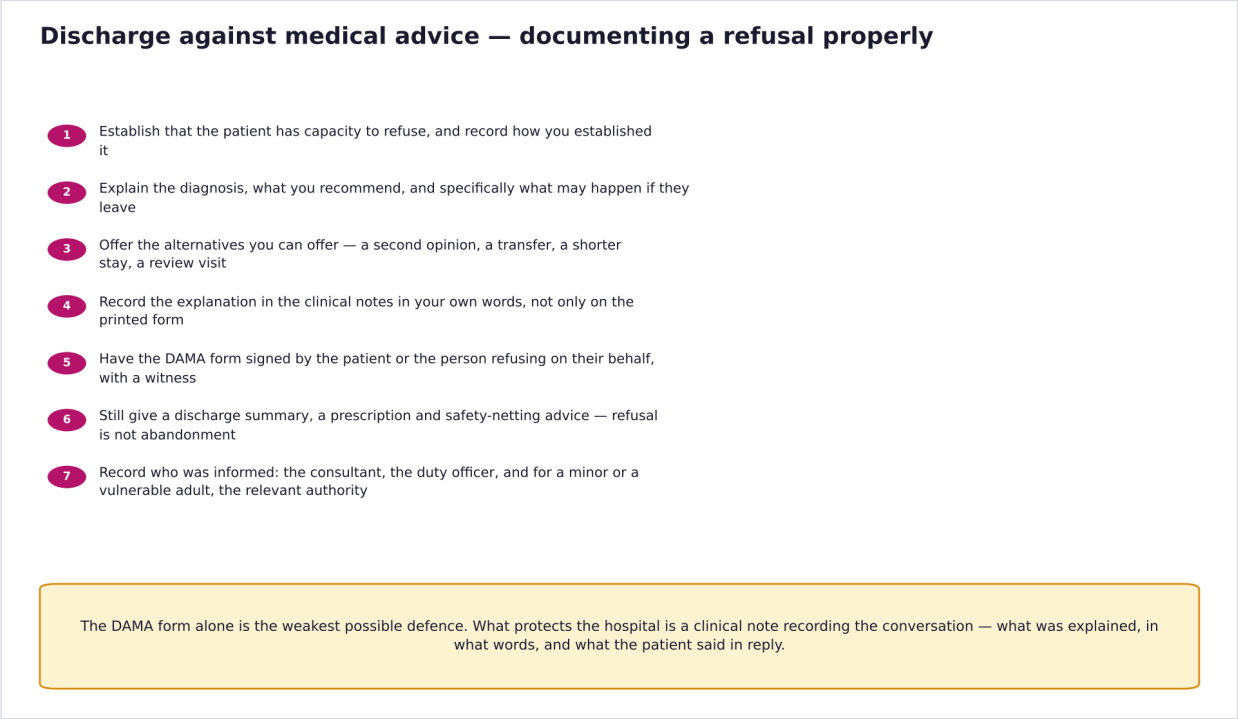


Figure 6.4 — Documenting a refusal and a discharge against medical advice.

A refusal by a patient with capacity must be respected, and the hospital's task is to ensure it is informed and to document it. The commonest failure is to treat the DAMA form as the whole of the documentation. It is the weakest part of it: a printed form signed under pressure, often in English, often not read.

- Record that capacity was present and how you satisfied yourself of that
- Record what you explained: the diagnosis, the recommended treatment, and specifically what may happen if the patient leaves or declines
- Record the alternatives offered — a second opinion, a transfer, a shorter stay, a review appointment
- Record the patient's own stated reason, in their words, because it frequently reveals a solvable problem such as cost or childcare
- Have the form signed, with a witness, and record who witnessed it
- Still provide a discharge summary, a prescription and specific return advice; a refusal does not end the duty of care
- Record who was informed: the treating consultant, the duty officer, and for a minor or a vulnerable adult, the appropriate authority

> **KEY POINT**
> Half of all DAMA departures in Indian hospitals are driven by cost, transport or family circumstances rather than by disagreement with the clinical plan. Recording the patient's stated reason frequently allows the hospital to solve the actual problem and keep the patient.

6.8 Consent in an electronic record

Where consent is captured electronically, the same elements apply, with two additions. The system must capture the patient's signature or authentication in a way that is bound to the specific document and cannot be transferred to another, and it must retain the version of the information document that the patient was actually shown. A hospital that updates its consent content without versioning cannot later show what the patient read.

6.9 Auditing consent

A consent audit that counts signatures will always report high compliance. A useful audit looks at a sample of forms against objective criteria and reports item-level failure.

Audit criterion	Pass requires
Correct form used	The current approved version, and the procedure-specific form rather than the general one
Procedure named in full	The actual procedure, not an abbreviation or a generic phrase such as ‘operation’
Side and site stated	Written in words where laterality applies, matching the site marking and the operation note
Risks section completed	Specific risks written or ticked, not left blank or struck through
Language and comprehension	The language of the form, or an interpreter named and signed
Patient signature and date	Present, dated, and timed before premedication
Clinician signature	Name, designation and registration number of the person who explained
Witness	Present where the form requires one, with name and relationship
Clinical note of the discussion	A corresponding entry in the clinical notes on the same date
Filed with the episode	In the correct file, with the procedure it authorises

6.10 Consent for anaesthesia

Anaesthesia is a separate intervention with its own risks and requires its own consent, taken by the anaesthetist or a clinician able to answer for it. A single surgical consent form that mentions anaesthesia in passing does not discharge the duty.

- The type of anaesthesia proposed, and the alternatives considered
- The material risks of that technique, including those specific to this patient’s comorbidities
- Risks of regional techniques discussed separately where regional anaesthesia is planned
- The plan if the technique fails or has to be converted
- Post-operative pain management, including any catheter or device that will remain
- Blood product use where it is anticipated, consented separately
- The pre-anaesthetic assessment entry recording that the discussion took place and when

6.11 Withdrawal of consent

Consent may be withdrawn at any time before or during a procedure by a patient with capacity, and the withdrawal must be acted on and documented. This arises most sharply in procedures performed under local or regional anaesthesia, where the patient is awake.

1. Stop at the earliest safe point. Where stopping immediately would itself cause harm, record what was done to reach a safe stopping point and why.
2. Record the time the patient withdrew consent, what they said, and who was present.
3. Record what was explained about the consequences of stopping at that point.
4. Record the clinical state at which the procedure was stopped and what remains to be done.
5. Record the plan: when and whether the procedure might be completed, and what interim management applies.
6. Record the discussion afterwards, once the patient is in a position to have it.

6.12 Consent, trainees and who operates

Patients are entitled to know who will perform their procedure. Where a trainee will operate, under supervision, that should be explained and recorded rather than discovered afterwards. The consent form should name the operator or the category of operator, and the operative note must name who actually performed each part.

> **CAUTION**
> A mismatch between the name on the consent form and the name in the operative note is a straightforward finding and a common one. Where the operator changes — a list is reallocated, a consultant is called away — the change should be explained to the patient and recorded before the procedure, not reconciled afterwards.

6.13 Research consent

Consent for research participation is governed by the ICMR National Ethical Guidelines and is separate from consent to treatment in every respect.

Element	Requirement
Ethics approval	The approval reference of the institutional ethics committee recorded, and the approved protocol version
Participant information sheet	The version-numbered sheet actually given to the participant, in a language they understand
Consent form	Version-numbered, signed and dated by the participant and by the person taking consent, with a witness where the participant cannot read
Copy to the participant	A signed copy given, and that recorded
Voluntariness	Explicit recording that participation is voluntary and that refusal will not affect clinical care
Re-consent	Where the protocol changes materially, fresh consent on the new version, recorded
Withdrawal	The right to withdraw recorded, and any withdrawal documented with what happens to data already collected

6.14 Photography, recording and teaching

Clinical photography and video are personal data and often identifiable. Consent must be specific about purpose, and the purposes must be separated: clinical record, teaching, publication, and any wider use each need their own permission.

- Record the purpose consented to, and do not extend it afterwards without fresh consent
- Record that the patient was told images may be irretrievable once published, where publication is contemplated
- Store clinical images within the hospital system, not on personal devices
- De-identify where the clinical purpose does not require identification, and record that this was done
- Record the right to withdraw consent and what withdrawal will and will not achieve
- Apply the same rules to recordings made by patients and families of clinical encounters, with a hospital policy stating the position

6.15 The consent conversation note

The single highest-value addition a hospital can make to its consent process costs about two minutes. It is a note in the clinical record, on the date consent was taken, recording the conversation. A workable template follows.

	Example entry
Consent discussion	**12/03, 16:40.** Discussed laparoscopic cholecystectomy with Mrs S and her son, in Hindi. Explained the diagnosis of symptomatic gallstones, the nature of the operation and the keyhole approach, expected hospital stay of one to two days, and recovery over two to three weeks. Risks discussed: bleeding, infection, bile duct injury including the possibility of further surgery, bile leak, conversion to open operation at a rate of about one in twenty, injury to adjacent organs, and anaesthetic risk, to be discussed further by the anaesthetist. Alternatives discussed: continued symptomatic management, with the risk of recurrent attacks, cholecystitis and pancreatitis. She asked about the scar and about returning to work; both answered. She wishes to proceed. Consent form completed and signed by her, before any premedication. — Dr R Menon, Consultant Surgeon, Reg. No. 54321

KEY POINT
If a consent dispute reaches a hearing, this note is what the hospital relies on. The form proves a signature; the note proves the conversation.

6.16 Putting this module to work

1. Pull five consent forms from last week and check each against the audit criteria above. Record item-level failures, not an overall pass rate.
2. Check whether any of those five has a corresponding clinical note of the consent discussion.
3. Check what time the patient signed relative to the premedication order.
4. Find out whether your hospital requires spousal consent for sterilisation. If it does, raise it, because the requirement has no legal basis.
5. Check whether your consent forms carry a version number, and whether the version in use matches the master list.

Knowledge Check

Work through each item before reading the model answer. These are applied questions: the answer is the standard of documentation your assessor, your medico-legal adviser or the next clinician to read the file would expect.

Q1. **A conscious, oriented woman is scheduled for a hysterectomy. The ward takes consent from her husband because he is the one paying and she defers to him. Is the consent valid?**

Answer: No. She is an adult with capacity, so her own consent is required and nobody else's substitutes for it. If she wishes her husband to be present for the discussion or to be involved in the decision, that preference should be recorded, and the discussion can involve him — but the signature and the decision must be hers. A hysterectomy performed on a husband's consent alone is performed without consent.

Q2. **A patient signs a consent form for a laparoscopic cholecystectomy on the trolley outside theatre, after premedication. The procedure is uneventful. Why is this still a problem?**

Answer: Valid consent requires capacity at the time of signing, and premedication compromises it. It also removes voluntariness in practice, because a patient on a trolley outside theatre is not in a position to decline. Even with a good outcome, the hospital cannot show that consent was validly obtained, and the practice will be a finding on any consent audit. Consent should be taken when the decision to operate is made and confirmed on admission, before any premedication.

Q3. **A surgeon finds an unexpected ovarian mass during a planned appendicectomy and removes it. The consent form named only appendicectomy. What should have been in place, and what should now be documented?**

Answer: The consent discussion should have addressed what would be done if an unexpected finding arose, and the form should have recorded the limits of what the patient authorised. Where the finding is not immediately life-threatening, the correct course is to complete the authorised procedure, close, and discuss the finding with the patient afterwards. Now, the operative note must record the finding, why the decision to proceed was taken, the clinical urgency relied upon, and the discussion with the patient as soon as she is able, together with the reason the additional procedure was not separately consented.

Q4. **A hospital includes HIV testing in its routine pre-operative panel without specific consent, and the result is filed in the general record. Which rules are breached?**

Answer: The HIV and AIDS (Prevention and Control) Act 2017 requires informed consent and pre-test counselling, both documented, before an HIV test, and restricts disclosure of the result. Testing without consent is unlawful; filing the result where it is visible to anyone handling the record compounds it with a confidentiality breach, which also engages the DPDP Act 2023. The hospital needs a specific consent and counselling process and restricted handling of the result.

Q5. **A patient with capacity refuses a recommended blood transfusion on religious grounds. What must the record contain, and what must it not contain?**

Answer: It must contain the assessment that the patient has capacity, what was explained including the specific risks of not transfusing, the alternatives offered such as cell salvage, iron or erythropoietin, the patient's own stated position, the signed refusal with a witness, who was informed, and the plan for continuing care. It must not contain pressure, repeated attempts recorded as persuasion, or any suggestion that the patient has been abandoned — the duty of care continues and the record should show it continuing.

Q6. **A consent audit reports ninety-six per cent compliance. On inspection, every form is signed but none has the risks section completed and none has a corresponding clinical note. What was the audit measuring?**

Answer: The presence of signatures, which is the least informative criterion available. A useful audit reports item-level results: correct form and version, procedure named in full, side and site, risks completed, language and interpreter, timing relative to premedication, identity and registration number of the explaining clinician, witness, a corresponding note in the clinical record, and correct filing. Reported that way, this hospital's compliance on the elements that matter is zero.

ASSESSMENT BANK
A fuller bank of 50 scored questions for this module is supplied separately as a JSON question bank for loading into your LMS.